جوستاف روسي انترناشيونال - مصر (EPE) *

طلب علاج أول مرة / متردد

إسم المريض رباعياً : ساره فتحي الهاشمي عبد المقصود
المحافظة : القليوبيه **الحي / المدينة :** شبرا الخيمه ثان **العنوان :** شبرا الخيمه ثان
رقم البطاقة : 28611230105221 **رقم التليفون :** 01273318820 **تاريخ تسجيل الإستمارة :** 15-10-2026
العمر(تاريخ الميلاد) : 23-11-1986 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : علاج مبادرة سرطان الثدي زولاديكس تحت الجلد لمده 6 شهور [14163] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: ساره فتحي الهاشمي عبد المقصود
National ID: 28611230105221

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي Goserelin (Zoladex) subcutaneous

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



Name: ساره فتحى الهاشمى عبدالمقصود	
MR. : 2118	

Detailed Medical Report

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من		بشأن تقرير طبي للمريض	
بقسم:		رقم طبي:	

نشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول :		تاريخ وساعة الخروج :	

وبعد الكشف والفحوصات والمتابعة وجد انه

The patient was diagnosed with metastatic breast cancer (MBC) and was placed on an aromatase inhibitor (Arimidex) with supplemental Zoladex. A new contrast-enhanced CT of the chest, abdomen, and pelvis (CTCAP) was ordered, with comparison to prior imaging, to obtain the necessary documentation for Herceptin approval.

Imaging review on 4/2026 demonstrated stable disease (SD) on the CTCAP, and the bone scan performed on 5/2026 was reported as free of metastatic involvement.

The patient's oncologic history includes right breast DCIS treated in 11/2021 with radiation therapy, mastectomy with reconstruction, and adjunct tamoxifen until 8/2022. Right axillary lymph node involvement was identified as invasive ductal carcinoma (IDC) with sentinel node positivity, leading to further surgery and radiation. In September 2022, the patient received one month of an aromatase inhibitor (Arimidex) followed by 12 weeks of Taxotere (Taxol) plus Herceptin, completing two cycles of Herceptin. By January 2023, repeat CT showed regression of the right axillary lymph node and a small S1 lytic lesion with sclerotic margins; the patient continued Herceptin for six additional cycles and completed 21 weeks of Taxol, achieving stable disease on a March 2023 CT. From April 2023 onward, the regimen comprised Arimidex, Zoladex, and Herceptin until Herceptin was discontinued in May 2024. A triphasic CT in 2024 identified a benign hepatic lesion, and bone scan showed no definite bone metastasis. Subsequent radiation therapy to the supraclavicular and supraclavicular lymph nodes, sentinel lymph node biopsy, and silicone reconstruction were performed for recurrent right axillary lymph node disease. The patient continued on AI plus Zoladex, port placement, and Herceptin through April 2026, with the most recent CTCAP on 4/2026 showing stable disease and a bone scan on 5/2026 remaining free of metastasis. The multidisciplinary team recommended continuation of the current treatment plan.

تحريرا في ,

التاريخ :	21-09-2026	الوقت :	

الطبيب المعالج : نهي سعيد علي محمد

يعتمد ,

NILE SCAN & LABS

مركز النيل للأشعة و التحاليل

Name : Sara Fathy Alhashmi Abdulmaqsood
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 02-05-2026 08:18
Medical No. : 364748

Tc-99m MDP FOLLOW UP SKELETAL SCINTIGRAPHY

History

40 year-old female patient, who gave history of surgically treated right breast cancer in 2021, received chemo-radiotherapy, on hormonal treatment and referred for follow up.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of 20 mCi of the tracer.

Findings

- ***Compared to the last bone scan dated 23/6/2025,*** the current scan revealed the same unchanged scintigraphic features with no evidences of osseous tracer uptake abnormalities that would account for bone secondaries.
- Degenerative features are depicted at multiple levels of the spine and big joints.

IMPRESSION

- The skeleton is keeping free from osseous deposits.

Much obliged
Dr. Alaa Nagy, MD,
Nuclear medicine

NILE DIAGNOSTIC CENTER

Call Center 19656
Bone Scan
[[DOCTYPE:BONE_SCAN]]

جوستاف روسي

Patient's Name: sara fathy elhashmy
Date: 2026-04-28
Referred by: Prof.Dr. cap+c
Hospital: Goustave Roussey International Egypt

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

***Compared to previous study dated 26/1/2026, the* current study reveled**

I- CHEST:

- Still noted bilateral scattered pulmonary atelectatic fibrotic bands
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.
- Clear right mastectomy operative bed with an implant.
- Right axillary LN is seen rounded measuring 13mm for US assessment.
- Clear left breast and left axilla.
- Right portacath is noted

II- ABDOMEN AND PELVIS:

- The liver shows homogenous parenchymal attenuation with still noted right hepatic lobe segment VI non-enhancing lesion measuring 8x6 mm. No dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses, diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections. IUCD is noted in place
- No abdominal or pelvic lymphadenopathy.
- No ascites.

III. SURVEYED SKELETON:

- No suspicious osseous lesions by CT FOR bone scan assessment.

Opinion:

- Stationary course since the last study except for the newly developed right axillary LN for US assessment.

Much Obliged;
Dr. Mai Shaaban MD

CT Scan
[[DOCTYPE:CT_SCAN]]

GUSTAVE ROUSSY
INTERNATIONAL
EGYPT

Name : sarah fathy el shamy	**Date : 26-01-2026 11:32**
Branch : Portable	**Medical No. : GRI-27930**
Ref.Physician Prof : cap+c	**Ins.Company :**

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN AND PELVIS REVEALED:

Unavailable old films

I- CHEST:

- Bilateral scattered atelectatic fibrotic bands
- No pulmonary nodules, masses or cavitary lesions.
- No consolidation or collapse.
- No hilar or mediastinal masses or lymph node enlargement.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.
- Unremarkable left breast.
- Clear right mastectomy operative bed with an implant.
- Right portacath is noted

II- ABDOMEN AND PELVIS:

- The liver shows homogenous parenchymal attenuation with a right hepatic lobe segment 6 non-enhancing lesion measuring 8x6 mm. No dilated biliary radicles.
- Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Normal C.T appearance of spleen, pancreas, suprarenal glands, aorta and IVC.
- The urinary bladder shows no evidence of gross masses, diverticulae or stones.
- Normal appearance of the pelvic organs with no pelvic masses or collections.
  IUCD is noted in place
- No abdominal or pelvic lymphadenopathy.
- No ascites.

III. SURVEYED SKELETON:

- No suspicious osseous lesions

Opinion:

- Clear right mastectomy operative bed with an implant.
- Right hepatic lobe lesion as detailed for further MRI assessment.

Much Obliged;

Dr. Zeinab Aly, MD

Patient Name	:	Request No	: 13003589
Patient No	: 28611230105221	Request Date	: 21/03/2024
Path No.	: 3029/24	Verification Date	: 01/03/2024
Sex	: Female	Financial Category	:
Age	: 37 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs Mayada Hegazy

Pathology Report

Clinical data
Right breast cancer.

Nature of specimen
Axillary lymph node dissection.

Gross specimen
Mulliple fibrofatty tissue pieces measred collectively 5x5x5 cm, entangling 28 nodules up to 4x3.5x1 cm.

Microscopic examination
Revealed tissue of lymph nodes showed deposits of invasive duct carcinoma GII in 6 lymph nodes (6/28) with focal extracapsular extension.

Diagnosis
Right breast cancer, Axillary lymph node dissection:
Deposits of invasive duct carcinoma GII ((6/28) with focal extracapsular extension.

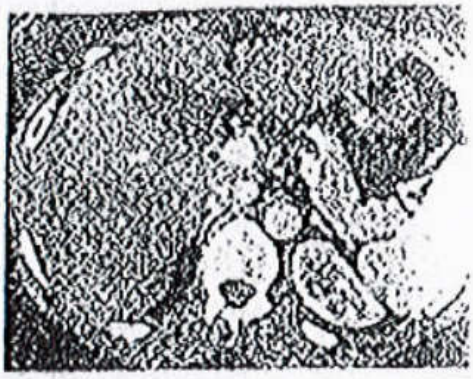
Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by :
DR.Menna Nabil

Room No. (4) - First Floor-Building No. 287- Nile Corniche - Mot 11 Qubeisi - Cairo, Egypt

Patient Name: Sara Fathy Alhashmy Abdel Maksoud Exam: C.T CHEST WITH CONTRAST Medical ID: 4140/2022	Date: 14/06/2022 Age: 38 Year(s) Pat. ID: 27930

- Average size of the heart is noted.
- The included lower neck cuts are unremarkable
- *A right porta-cath is noted.*
- *Right breast silicone implant is noted with no definite complications.*



- *The scanned upper abdominal cuts reveal mildly enlarged liver with diffuse decreased hepatic attenuation denoting diffuse fatty hepatomegaly, for correlation with lipid profile.*

CONCLUSION: -

- Unremarkable CT examination of the chest.
- Right breast silicone implant with no definite complications.

Much Obliged;

Dr.Mona Ali,MD

A.Prof. of Radiology Ain Shams University

CT Scan
[[DOCTYPE:CT_SCAN]]

Name : Sara Fathy Alhashmi Abd Almaqsood
Branch : Maadi
Ref.Physician Prof : Hrull Hospital

Date : 23-06-2025 06:15
Medical No. : 134614
Ins.Company : (تأمين الصحي)

$Tc^{99}m$ MDP SKELETAL SCINTIGRAPHY

History:

- A 38-year-old lady, with history of right breast cancer, referred for metastatic work up.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- No abnormal active or cold focal bony lesions could be seen over the surveyed skeleton to account for metastatic bone disease.
- Degenerative changes are seen at the spine and big joints.

Impression:

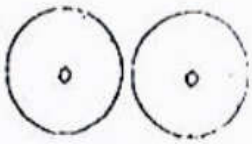
- The skeleton is scintigraphically free from metastases.

Much obliged
Dr. Alaa Nagi, MD
Consultant of nuclear medicine

NILE DIAGNOSTIC CENTER

MDT form

A-Patient ID:
1- Name : [redacted] Sex: Female
3- Medical number: 83101967
Old Med : [redacted]
4149/2022
2-ID number:
Patient Age : 38 Y



Family history	[redacted]	Phone	[redacted] 01227338820	BMI	[redacted]	REASON OF MDT	[redacted]
marital status	[redacted]	NRS Score	[redacted]	GROUP	[redacted]	Date of MDT	[redacted] 11/01/2025
Co morbidity	[redacted]	Menopausal status	[redacted]	Initial STAGE	[redacted]		

Clinical data

Breast Imaging :

Mets Workup :

BX	ER	PR	Her 2 Neu	SISH	Ki67
IDC	8/8	8/8	3		

T	N	M
	1	

Sonomammography

PATIENT HISTORY

11/2021: rt breast DCIS-->RT MRM & REConstruction--> adj. tamoxifen till 8/2022
Rt axillary LN-->IDC+RT SC LN-->FREE ---> BX
9/2022 : oligomet received 1 month AI (aramidix) -->12 wks taxol +herceptin (2 cycles)
1/2023:new ct: regressive course of Rt axillary LN & small S1 lytic with sclerotic margin
continue G herceptin + RECEIVED 21 WKS TAXOL 28/3/2023CT-->SD
4/2023 aramidix+ zoladex+ herceptin till now , herceptin stopped
5/2024 ct triphasic : hepatic benign lesion MRI, bone scan: no definite BM infiltration
RT SSM+SLNB +silicone: recurrent rt Axillary LN 6/28
pt received AI+zoladex -->NO PORT, no anti her2
CT CAP 9/2024 : FREE

MDT OUTCOME & PLAN

FOR AI + ZOLADEX + PORT
CONTINUE HERCEPTIN IV

SIGNATURE

MEDICAL ONCOLOGY	[redacted]	RADIOLOGY	[redacted]
SURGERY		PATHOLOGY	



National ID – Front
[[DOCTYPE:ID_FRONT]]



National ID – Back
[[DOCTYPE:ID_BACK]]